



THE CLARITY DIAGNOSTIC

The Mountain That Hides Itself

Network summary across five locations.
The patterns. In order of impact.

Mountainside Dental Group — Network Summary | Southern California

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I spent some time with Mountainside Dental Group's full digital footprint this week — five location pages, five Google Business Profiles, five Maps markets, the social accounts, the legacy domain, the blog, the founder story. Here's what stood out across the network.

Mountainside Dental Group is one of the most quietly substantial independent dental networks in Southern California. Five locations. Three generations of Loma Linda dental family. Twenty-two years of deliberate, no-DSO growth from Dr. John Gillian's Yucaipa practice to a five-office regional group. Twenty-five thousand patients, give or take. A flagship in La Quinta — 786 Google reviews at 4.9 stars, nine specialists under one roof, the kind of bench depth most independent groups never reach. An active blog publishing location-specific content into 2026. A real paid-ad stack. The clinical and operational infrastructure of a network that's earned its position over two decades.

Twenty-two years. Five locations. Twenty-five thousand patients. A digital footprint that consistently understates every one of those numbers.

The gap is between what the network has built and what its digital footprint actually says about it. The homepage's trust counter reads "5,000+ Happy Patients" — a five-times undercount of the network's own claimed scale. Two of the five location pages tell two different founding stories on the same page: Lake Forest's hero says "since 2024" while the body copy says "since 2018"; Rancho Santa Margarita says "since 2021" in the hero and "since 2014" in the schema. The largest social account in the network — La Quinta's Instagram — has 62 followers across 124 posts. The 2004 original location in Yucaipa has fewer Google reviews than a younger competitor with a confusingly similar name in the same town. None of these is fatal on its own. Together they tell a consistent story.

5x

PATIENT-COUNT UNDERCOUNT ON THE HOMEPAGE TRUST COUNTER

"5,000+" displayed; 25,000+ claimed by the network

The five per-location diagnostics in this package go deep on each market. This network summary covers the patterns that show up across the locations, the parts of the owner story that unify the network, and the sequencing question — where to start, when you have five locations and one team to fix them.

How I See This

The thing that strikes me about Mountainside is that the hard part — the part most networks fail at — is already done. Dr. Wolfram has spent 22 years building this independently. No DSO money. No private equity carve-out. A three-generation Loma Linda dental family. A flagship that operates as a true specialist hub, not just a generalist office with a sign that says "specialists welcome." A blog that's actually publishing location-specific content. Acquired practices in Lake Forest and Rancho Santa Margarita that brought existing patient bases and 25-plus-year clinical credentials into the network. The work is real, and the patients clearly know it.

The hard part is done. The clinical scale exists. The reputation exists. The growth path was deliberate. What's missing is the digital system that lets the rest of the world see all of it.

But the network's digital footprint doesn't reflect any of this. That's the cross-cutting pattern. Five separate Facebook pages with eight to thirty likes each. One Instagram account with 62 followers. A homepage trust counter set to a fifth of the actual patient base. Two location pages that disagree with themselves about when the practice opened. A founder-story page that lives one click away from the location pages that need it most. A 2004 original location that ranks behind a younger competitor in its own founding town. A heavy paid-ad stack — Google Ads, Facebook Pixel, ClickCease, Microsoft Clarity — driving every click to a Gravity Forms callback funnel rather than a real-time booking widget. The legacy `drwolfram.com` site still live, with an SSL error, competing for branded searches against the parent brand.

This is what I'd call **the Mountain That Hides Itself** problem. The clinical authority is the mountain. The digital footprint is the haze that keeps it invisible to anyone driving by. The locations all share the same haze pattern — Yucaipa is the part of the mountain most buried because that's where the longest history should be most visible. La Quinta is the network's tallest peak and even the peak is partly buried. Rancho Mirage is confused with the next ridge over. RSM and Lake Forest are the parts where the rebrand actually shortened the mountain — claiming "since 2021" or "since 2024" on practices with longer histories underneath them. The gap is consistent across all five.

The blog is the bright spot. Active publishing into 2026. Location-specific content. Real SEO discipline. The content engine works. It's the distribution layer — social, GBP, hero copy, founding-date integrity — that's not letting the engine produce.

What makes this tractable is that none of it requires building from scratch. The infrastructure exists. The clinical reputation exists. The patient base exists. The brand promise — *"Where we Believe in HONEST Dentistry"* — is the right one for a 22-year independent network. What's missing is the system that closes the gap between what the network has earned and what its digital footprint communicates. The findings below are five cross-cutting patterns and two location-specific standouts. Each per-location diagnostic in this package goes deeper on the local detail.

The Visual Scorecard

This is the page Dr. Wolfram should be able to scan in 30 seconds and immediately know which location has which gap, and which gaps are network-wide. The columns are the five locations. The rows are the patterns. Each cell is a brief observation, not a grade.

Pattern	Yucaipa (2004)	La Quinta (2020)	Rancho Mirage (2021)	RSM	Lake Forest
Maps top-3 in own city	Not in top 3	#1 or top 3	Rank 3-5	Rank 3-5	Rank 2-3
Google review volume	~129 (vs. local 829)	786	250-267	148	159-250
Founding date integrity	Consistent (2004)	Consistent (2020)	Consistent (2021)	Hero 2021 / body 2014	Hero 2024 / body 2018
GBP description optimization	Generic	Doesn't lead with 9-specialist roster	Doesn't reference Bob Hope Dr / re-tiree market	Doesn't lead with Dr. Welcome's USC + 28 yrs	Doesn't lead with Harvard implant credential
Real-time online booking	None (callback form)	None (callback form)	None (callback form)	None (callback form)	None (callback form)
Per-location social activity	FB ~8 likes; no IG	FB ~30 likes; IG 62 followers	FB exists; no IG	FB exists; no IG	FB exists; no IG
Hours coverage	7am Th opens (unique); F 8-3	M-Th 8-5; F 8-2	M-Th 8-5; F 8-2	M-Th 8-5; F closed	M-Th 8-5; F 8-2
Doctor names featured	None on location page	Specialists in reviews	Dr. Castillo / Dr. Deborah in reviews	Dr. Welcome in title	Drs. Chang & Azimi in title
Service-specific page depth	Generic family / cosmetic	"9 dentists" claim, no per-specialty pages	Prosthetic emphasis underdeveloped	Endo/perio focus underdeveloped	Pediatric/ortho/Invisalign + same-day, hero buries it

0/5

LOCATIONS WITH REAL-TIME ONLINE BOOKING

Every paid click and every organic visitor on the network ends at a callback form

Five locations. Five different versions of the same gap. The network needs one consistent fix applied five times more than it needs five different strategies.

There's no grade here on purpose. The point isn't to rank Wolfram's network against itself. The point is to show that most of these patterns are network-wide, not local — and that the fix at the network level is the leverage point. The per-location diagnostics handle the local nuances.

The Cross-Cutting Findings

Five patterns that show up across the network. Each is detailed below, with effort tags. The per-location diagnostics reference these and apply them to local context.

CROSS-CUTTING FINDING 1: THE 5X PATIENT-COUNT UNDERCOUNT

The homepage trust counter reads **"5,000+ Happy Patients."** The network claims 25,000+ patients across five locations. This is the single clearest piece of evidence that Mountainside undersells itself: the network's most prominent first-impression number on its own homepage is set to a fifth of reality.

HOMEPAGE TRUST COUNTER

5,000+ Happy Patients

NETWORK ACTUAL CLAIMED

25,000+ patients

The number that should anchor the network's authority is set to one-fifth of it.

Why this matters

80%

OF THE NETWORK'S ACTUAL PATIENT VOLUME INVISIBLE ON ITS OWN HOMEPAGE

A trust signal that should compound across 22 years is set to a single year's worth of growth

A trust counter on a network homepage is doing one of two jobs: it's either telling a prospective patient "this is a real practice with a real history" or it's telling them "this practice doesn't track its own scale." A 5,000+ counter for a 25,000+ network does the second. For a sophisticated patient researching specialists in the Coachella Valley or Orange County, the counter undermines the case the rest of the page is trying to make.

This isn't just a copy-edit issue. The 5,000 number traces to a schema definition somewhere in the WordPress build that nobody updated when the network grew. The pattern repeats across the digital infrastructure — the founding dates, the GBP descriptions, the social-account follower counts. The system was built for a smaller version of the practice and hasn't been told the practice grew.

Here's what I'd do

Confirm the actual patient count this week. Pull a count out of the practice management software across all five locations. Twenty-five thousand may be conservative or generous; the actual number is the right number. Whatever it is, that's what the homepage should say.

Update the homepage trust counter to the actual number this week. This is a 30-minute developer task on WordPress. The counter is in a Mountainside-specific custom plugin (mountainside-dental-pages v3.1.0) — the patient-count value lives in a single setting.

Audit the rest of the schema for similar gaps. The 25,000 vs. 5,000 conflict on the homepage suggests the schema-level data hasn't been kept in sync with the practice's growth. While the developer is in the file, audit organization-level claims (employees, locations, founding date) and patient-level claims (patient counts, review counts) for similar drift.

Effort: Quick win (homepage counter and schema audit: half a day combined).

CROSS-CUTTING FINDING 2: THE CALLBACK FUNNEL — 5 OF 5 LOCATIONS

All five /appointments-{location}/ URLs route to the same Gravity Forms instance. There is no real-time scheduling integration anywhere in the network. The CTA copy reads, verbatim: *"Request an Appointment — Takes less than a minute. We'll contact you to confirm your visit."* The full ad stack is firing — Google Tag Manager (GTM-TGQJPCPW), Google Ads conversion tracking (AW-773336714),

Facebook Pixel (1154824026278843), Microsoft Clarity (tq4efvcwqp), ClickCease anti-click-fraud — and every paid click and every organic visitor on the network lands on a form that says "we'll call you back."

The network is paying to drive five locations' worth of traffic to a callback form. Every dollar of ad spend ends with the same 'we'll contact you' message a one-location practice would use.

Why this matters

\$0

REAL-TIME BOOKINGS COMPLETED ON THE NETWORK SITE

Across five locations, full ad-stack tracking, 25,000-patient base, and an active blog

Three things compound here. First, real-time booking is no longer a competitive advantage in dental — it's a baseline expectation for a network of this scale, particularly in affluent Coachella Valley and Orange County markets where competitors increasingly offer self-scheduling. Second, the cost of the gap rises with traffic volume — and Mountainside is *paying* to drive traffic to it, which means the friction is being purchased at full retail. Third, the friction shows up at the moment of highest patient intent — the patient has already decided to book and is then asked to wait.

The current callback-first design is consistent with how a single relationship-driven practice operates: phones answered by people who know the schedule, complex insurance verification done in conversation, complicated cases triaged before the appointment is set. That's a real operational reason. But it doesn't have to be all-or-nothing. A real-time booking widget can be configured to handle straightforward cleanings and recall appointments while routing complex cases to the callback flow. The practice doesn't have to abandon its phone-first approach to capture the bookings that are leaving on the table tonight.

Here's what I'd do

Pilot real-time booking at La Quinta first (the highest-traffic location). Pick a platform that integrates with the existing practice management software — NexHealth, LocalMed, and Dental Intelligence are the leading options for U.S. multi-location independents. Configure it to handle cleanings, recall appointments, and emergency exam slots. Route complex cases (full mouth restorations, implant consultations, second opinions) to a callback queue that the front desk handles the way they always have.

Measure the lift over 90 days. Compare booked-appointment volume during the pilot window against the prior 90 days at La Quinta. The metric that matters is *new patients booked outside business hours* — the patients the callback funnel was structurally losing.

If the lift justifies it, roll out to the other four locations sequentially. Yucaipa, Rancho Mirage, Lake Forest, RSM, in that order — Yucaipa next because the home-market visibility gap is the highest-priority location-specific issue and online booking helps close it.

Effort: Medium (pilot platform integration + measurement: 30 days; full network rollout: 60-90 days).

CROSS-CUTTING FINDING 3: THE FOUNDING-DATE DRIFT (RSM AND LAKE FOREST)

Two of the five location pages tell two different founding stories on the same page. Rancho Santa Margarita's hero subtitle says "since 2021"; the schema description says "since 2014" — a 7-year gap. Lake Forest's hero says "since 2024"; the schema says "since 2018" — a 6-year gap. The likely explanation in both cases is that the location was an existing practice acquired into Mountainside (Dr. Welcome's RSM practice in 2021; Drs. Chang and Azimi's Lake Forest practice apparently around 2024) and the rebrand overcorrected the founding date — claiming the *Mountainside* tenure rather than the *practice* tenure on the hero, while the body copy retained the original.

Two acquired practices, telling two different stories about themselves on the same page. The brand promise is Honest Dentistry. The hero contradicts the body copy.

Why this matters

13 years

COMBINED PRACTICE HISTORY THROWN AWAY BY THE TWO HERO SUBTITLES

RSM (2014) and Lake Forest (2018) tenure that the rebrand erased

The brand promise of the entire network is "*Where we Believe in HONEST Dentistry.*" Two of five location pages contradict that promise on themselves. A patient who reads carefully — and the patients who read carefully are usually the high-LTV ones — sees the inconsistency. A patient who reads only the hero loses six or seven years of practice history that should be the credibility signal.

The math is also wrong. Dr. Welcome has practiced in Rancho Santa Margarita since 2014. That's 12 years of patient relationships, 12 years of community presence, 12 years of accumulated word-of-mouth.

The hero subtitle erases all of it. Same at Lake Forest — Drs. Chang and Azimi have eight years of patient history in the Lake Forest market. The hero claims two.

Here's what I'd do

Fix Lake Forest this week. Update the hero subtitle to "since 2018" — the date in the body copy. If "since 2024" was intentional to signal Mountainside's involvement, find another way to do it ("Joined the Mountainside Dental Group in 2024" can sit elsewhere on the page; the founding-date claim should be the original).

Fix RSM this week. Same approach. Hero subtitle to "since 2014." Note Dr. Welcome's 28-year career or the 2021 Mountainside acquisition separately, where they belong.

Audit the rest of the network for similar drift. Yucaipa's "since 2004" matches; La Quinta's "since 2020" and Rancho Mirage's "since 2021" appear consistent. But the same WordPress plugin that produced the conflicts at RSM and Lake Forest may be producing similar inconsistencies elsewhere — copyright dates, schema references, internal cross-page claims.

Effort: Quick win (two-page edit: 1 hour; audit the rest of the network: half a day).

CROSS-CUTTING FINDING 4: THE FIVE FACEBOOK PAGES AND 62-FOLLOWER INSTAGRAM

Five separate Facebook pages — Yucaipa (8 likes), La Quinta (30 likes), Rancho Mirage, RSM, Lake Forest. Combined Facebook engagement: under 100 likes for a 22-year, 25,000-patient network. One identifiable Instagram account: @msidentlaquinta, 62 followers, 124 posts. The footer of mountainside-dental.com links to generic @mountainsidedental social handles that don't appear to map to active accounts. No consolidated network-level Instagram. No active TikTok or YouTube. The blog is the only network-level content channel that's actually publishing.

786

LA QUINTA GOOGLE REVIEWS

62

LA QUINTA INSTAGRAM FOLLOWERS

The network's flagship has the audience to broadcast to. The channel is dormant.

Why this matters



<100 likes

TOTAL FACEBOOK ENGAGEMENT ACROSS 5 PAGES

A network claiming 25,000 patients presents a social footprint smaller than a six-month-old solo practice

A patient researching a dental practice cross-channel — Google, then Yelp, then Facebook, then Instagram — sees the practice's authority refracted across the platforms. When the Google footprint is strong (4.9 stars across the network) and the social footprint is empty, the disconnect itself becomes a signal. The patient's read is something like *"Either this practice is bigger than they show or smaller than they show — and I can't tell which."* Either read costs the practice the booking.

This isn't a problem of the practice having nothing to say. The blog publishes location-specific content monthly. The clinical work generates real before-and-afters. The team is working in five offices. The content exists. The distribution layer doesn't.

Here's what I'd do

Consolidate to one Instagram and one Facebook account, both at the network level. Each post can be geo-tagged to the relevant location and tagged with the staff present. The five separate Facebook pages can be redirected (or merged via Facebook's tools) to the consolidated page. The fragmented audiences are cumulatively small enough that consolidation costs almost nothing.

Restart the consolidated Instagram with a low-overhead format. A 30-minute monthly photo session per location (rotating through the five), staff intros, before/after with patient consent, the 9-specialist roster at La Quinta featured one specialist at a time, the founder story at Yucaipa, the same-day care promise at Lake Forest. The blog already produces topical content; the social channels can amplify and visualize what's already being written.

Pilot one Instagram Reel or short video per month from the most distinctive location — typically La Quinta given the specialist depth, but Yucaipa's founder story (three generations, Loma Linda, Dr. Gillian acquisition) is an underused reel topic for the original location.

Effort: Medium (consolidation + cadence setup: 30 days; ongoing: 30-minute monthly per location).

CROSS-CUTTING FINDING 5: GBP SETUP WITHOUT NETWORK COORDINATION

Each of the five Google Business Profiles is configured as if it were a stand-alone practice, not part of a coordinated network. Primary categories vary across locations (Cosmetic Dentists vs. General Dentistry). Business name formats vary ("Mountainside Dental Group - La Quinta" vs. "Mountainside Dental Group of Lake Forest"). Photo counts cluster around 22-23 per location — consistent across the network, but consistently below industry benchmarks of 75+ for an established practice. No coordinated

GBP posting strategy at any location. No active Q&A management. No consistent description template that surfaces the network-level brand promise (HONEST Dentistry, the 9-specialist roster at La Quinta, the founder's three-generation Loma Linda heritage at Yucaipa).

Five GBP listings managed as if they were five separate practices. None of them benefits from being part of a network.

Why this matters

~22 photos per location

GBP PHOTO COUNT ACROSS THE NETWORK

Below the 75+ benchmark for an established practice on a single platform that drives the most local visibility

Google Business Profile is the most direct conversion path from "patient searches" to "patient walks in." For a single practice, GBP optimization is a baseline. For a network of five locations, the GBP layer is also where the network's brand consistency gets stress-tested — and right now, the five GBPs look like they were set up independently in 2020 and have been left alone since. The network advantage (one team, one brand, shared content) isn't visible at the GBP layer.

The Maps ranking pattern across the network reflects the GBP setup. La Quinta dominates its market — the GBP is well-tended at the flagship. Yucaipa is out of the top 3 in its founding town. Rancho Mirage ranks 4-6 on implant searches behind 30-year single-specialty veterans. RSM and Lake Forest sit mid-pack. The GBP layer is the single most leveraged place to address visibility variance across the network.

Here's what I'd do

Standardize the GBP business-name format across all five locations. Pick one (e.g., "Mountainside Dental Group - [City]") and apply it consistently. Inconsistent name formats create confusion in branded searches.

Standardize the primary GBP category across all five. Cosmetic Dentists vs. General Dentistry vs. Family Dentist makes a real difference in which queries surface the location. Pick one primary category that fits the network position and apply it everywhere; let the secondary categories handle local nuance.

Bring photo counts up to the 75+ benchmark across all five. A 30-minute photo session per location, rotated quarterly, will get there inside a year. The blog content engine already produces some imagery

— pull from there as a starting point.

Activate GBP posting on a monthly cadence at all five locations. Each location's GBP can post the same network-level content (founder story, 9-specialist roster, blog post links) plus location-specific content (staff intros, local community involvement). The blog provides the source material; the GBP post is the distribution.

Audit each GBP description on the same template. Lead with the network's brand promise. Substantiate the location-specific claim (specialist roster, named doctors, market position). Include the keywords that match the local Maps queries the location is competing for.

Effort: Medium (template setup: 1-2 days; per-location application: 30 minutes each; ongoing posting cadence: 30 minutes per month per location).

Two Location-Specific Standouts

The five per-location diagnostics in this package go deeper on each location's market and findings. Two of the five deserve a brief mention here as the locations doing something the rest of the network should learn from, or struggling in a way the rest of the network should look at carefully.

STANDOUT 1: LA QUINTA IS THE NETWORK'S PROOF THAT THE SYSTEM WORKS

La Quinta has 786 Google reviews at 4.9 stars. That's 47 percent of the network's total review volume in a single location. The reviews mention specific specialists — Dr. Huh, Dr. Wolfram, Angelica the hygienist — and the responses to those reviews are personalized. Google ranks the location #1 or top-3 on every meaningful Coachella Valley search. The 9-specialist roster is a verifiably unique market position; no direct competitor in the immediate Coachella Valley matches the depth.

This proves the network *can* execute at scale. The review-acquisition system, the response cadence, the specialist-named patient experience that converts to specialist-named reviews — all of it works at La Quinta. The opportunity at the network level is replicating the La Quinta playbook across the four other locations. Yucaipa has 22 years and 129 reviews; La Quinta has 6 years and 786. The difference isn't tenure — it's the system around the patient relationship.

The detailed diagnostic on La Quinta (`mountainsidelaquinta_clarity_diagnostic.md`) covers the gaps inside La Quinta itself — the GBP doesn't lead with the 9-specialist claim, the Instagram is dormant, the specialist-named reviews aren't being syndicated outside Google. The location is the network's flagship and also the place where the most leverage sits.

STANDOUT 2: YUCAIPA IS THE NETWORK'S HARDEST CONVERSATION

Yucaipa is the founding location. 2004. The acquired Gillian practice. The founder's hometown. The three-generation Loma Linda dental family lives in this market. And yet it has the fewest Google reviews

in the network (~129 cross-platform), is out of the top 3 for "Yucaipa dentist," and is being out-shouted by a competitor with a confusingly similar name (*Yucaipa Dental* — 5.0 stars, 829 reviews). The 22-year tenure that should be the network's loudest authority signal at this location is its weakest visibility position.

There's a pattern here that recurs across networks I audit: the founding location, which has the longest history and the strongest emotional investment, is often the worst-tended digitally. The newer locations get the templates, the photos, the SEO discipline. The original gets taken for granted. Yucaipa fits the pattern.

The Yucaipa diagnostic in this package goes deep on the local detail — the specific competitor positioning, the founder-story page that lives one click away, the 7am Thursday opens that no other location offers and that the GBP doesn't surface. The network-level point: Yucaipa is the location where the "mountain that hides itself" pattern is most visceral, and where the longest-tenure asset in the network is being taxed by neglect rather than leveraged as authority.

The Owner Story (Told Once, Here)

This is the part of the diagnostic that doesn't appear on any of the five per-location pages — by design. The owner story is a unifying thread across the network, not a per-location asset, and it deserves to be told once, in full, in this network summary.

Dr. Steve Wolfram was born and raised in the Yucaipa-Loma Linda area. His father is a longtime dentist who still serves as faculty at the Loma Linda University School of Dentistry — the institution at the center of one of the most distinct dental traditions on the West Coast. That makes Dr. Wolfram a third-generation member of a Loma Linda dental family. He followed his father's path: Loma Linda School of Dentistry, then a stretch of practice in Oregon, then a return home with a clear vision. The clinical work is his to deliver. The practice was his to build.

In 2004, he acquired Dr. John Gillian's Yucaipa practice as the original Mountainside location. That's the first chapter — a young dentist returning to his hometown to take over an established community practice from a respected predecessor. Yucaipa is where the network started, and the practice has now operated continuously at that location for 22 years.

The growth path from Yucaipa to five locations was deliberate, not opportunistic. Dr. Wolfram did not take DSO money. He did not roll the network up into a private-equity carve-out. The network is family-owned, single-owner, no outside capital. That's an unusual decision for a dental network of this size in 2026. Most independent groups that get to five locations have either taken outside investment or sold to one of the consolidators by year fifteen. Mountainside hasn't.

Five locations. Twenty-two years. Three generations of Loma Linda dental tradition. No DSO. No private equity. The story most networks of this size can't tell anymore.

The five locations followed in order — La Quinta in 2020 as the Coachella Valley hub with nine specialists under one roof; Rancho Mirage in 2021 as the affluent retiree-market satellite; Rancho Santa Margarita acquired in 2021 (Dr. Welcome's 12-year practice integrated into the network); Lake Forest acquired more recently (Drs. Chang and Azimi's Harvard-trained implant practice). Twenty-five thousand patients across the network, give or take. A flagship that operates at specialty depth. Acquired locations that brought existing patient bases and 25-plus-year clinical credentials.

The reason this story matters to the diagnostic is that the network's positioning lives or dies on it. *"Where we Believe in HONEST Dentistry"* is the right brand promise for a 22-year independent network with a three-generation Loma Linda heritage. It's the wrong brand promise for a network whose pages disagree with themselves about when they opened. The founding-date conflicts at RSM and Lake Forest aren't typos — they're a brand-promise problem. The Yucaipa visibility gap isn't just a SEO problem — it's a story-distribution problem. The 5,000-vs-25,000 patient counter isn't just a copy edit — it's a network-scale problem.

The fixes spelled out in the cross-cutting findings above are the mechanics. The story is the why.

How I'd Sequence the Fixes Across the Network

The locations all share most of the cross-cutting patterns, so the sequencing is mostly network-level for the first 30 days, then location-specific for days 30 to 90.

This week

- Update the homepage trust counter from "5,000+" to the audited actual patient count. (Cross-cutting Finding 1.)
- Fix Lake Forest's hero subtitle to "since 2018" and RSM's to "since 2014." (Cross-cutting Finding 3.)
- Audit the WordPress schema across all five location pages for similar founding-date or scale drift.
- Pick the real-time booking platform for the La Quinta pilot and start the integration. (Cross-cutting Finding 2.)

Next 30 days

- Standardize GBP business names, primary categories, and description templates across all five locations. (Cross-cutting Finding 5.)
- Bring GBP photo counts up to the 75+ benchmark at all five locations. (Cross-cutting Finding 5.)
- Resolve drwolf.com — either redirect to mountainside-dental.com/yucaipa/ or fix the SSL and consolidate.
- Consolidate social to one Instagram and one Facebook account. Restart the cadence at the network level. (Cross-cutting Finding 4.)
- Launch the La Quinta real-time booking pilot.

30-90 days

- **Yucaipa:** Launch a review-acquisition system to close the gap with Yucaipa Dental. (See mountainsideyucaipa_clarity_diagnostic.md.)
- **La Quinta:** Build out specialty service pages and start syndicating specialist-named reviews to bio pages. (See mountainsidelaquinta_clarity_diagnostic.md.)
- **Rancho Mirage:** Reposition as the Coachella Valley prosthodontic and full-mouth restoration center; differentiate from La Quinta. (See mountainsideranchomirage_clarity_diagnostic.md.)
- **RSM:** Multi-platform review-acquisition system; route patients to Yelp + Google + Healthgrades, not just Google. (See mountainsidersm_clarity_diagnostic.md.)
- **Lake Forest:** Hero, meta description, and GBP rewrite leading with same-day care + Harvard implant credentials. (See mountainsidelakeforest_clarity_diagnostic.md.)
- Roll out real-time booking to the four locations not in the pilot, in order: Yucaipa, Rancho Mirage, Lake Forest, RSM.

Ongoing

- Monthly blog cadence (already happening — keep it going).
- Monthly GBP posts at all five locations on a coordinated network template.
- Monthly review velocity check at each location.
- Quarterly Maps ranking audit on the five key search terms in each market.

- Quarterly photo refresh per location (one 30-minute session per quarter, rotated through the five offices).
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Cross-Document Reference Map

The five per-location diagnostics in this package:

- mountainsideyucaipa_clarity_diagnostic.md — *The Mountain Started Here. The Market Doesn't Remember.* — The original 2004 location, weakest market visibility, Yucaipa Dental competitor dominance.
- mountainsidelaquinta_clarity_diagnostic.md — *The Network's Strongest Practice. Almost Invisible on the Searches That Pay.* — The flagship, 786 reviews, 9-specialist roster, dormant Instagram.
- mountainsideranchomirage_clarity_diagnostic.md — *Bob Hope Drive Is the Story. The Page Doesn't Use It.* — The Coachella Valley satellite, 30-year implant veterans owning the market, geography underplayed.
- mountainsidersm_clarity_diagnostic.md — *A Practice With Two Birthdays on the Same Page* — The 7-year founding-date gap, Dr. Welcome's USC + 28-year credentials, 10x Yelp/Google review platform discrepancy.
- mountainsidelakeforest_clarity_diagnostic.md — *A Practice That Has Been Here Since 2018. The Hero Says 2024.* — The 6-year founding-date gap, Drs. Chang and Azimi's Harvard implant training buried, same-day care promise unsurfaced.

Each per-location diagnostic stands on its own. The cross-cutting patterns above (callback funnel, founding-date drift, GBP coordination, social fragmentation, patient-count undercount) are referenced briefly in each per-location document with a pointer back to this network summary for the full mechanism.

Other Things I Noticed

How I'd Sequence the Fixes

This week

- Update the homepage trust counter from "5,000+" to the audited actual patient count. (Cross-cutting Finding 1.)
- Fix Lake Forest's hero subtitle to "since 2018" and RSM's to "since 2014." (Cross-cutting Finding 3.)
- Audit the WordPress schema across all five location pages for similar founding-date or scale drift.
- Pick the real-time booking platform for the La Quinta pilot and start the integration. (Cross-cutting Finding 2.)

Next 30 days

- Standardize GBP business names, primary categories, and description templates across all five locations. (Cross-cutting Finding 5.)
- Bring GBP photo counts up to the 75+ benchmark at all five locations. (Cross-cutting Finding 5.)
- Resolve `drwolfram.com` — either redirect to `mountainside-dental.com/yucaipa/` or fix the SSL and consolidate.
- Consolidate social to one Instagram and one Facebook account. Restart the cadence at the network level. (Cross-cutting Finding 4.)
- Launch the La Quinta real-time booking pilot.

30-90 days

- ****Yucaipa:**** Launch a review-acquisition system to close the gap with Yucaipa Dental. (See ``mountainsideyucaipa_clarity_diagnostic.md``.)
- ****La Quinta:**** Build out specialty service pages and start syndicating specialist-named reviews to bio pages. (See ``mountainsidelaquinta_clarity_diagnostic.md``.)
- ****Rancho Mirage:**** Reposition as the Coachella Valley prosthodontic and full-mouth restoration center; differentiate from La Quinta. (See ``mountainsideranchomirage_clarity_diagnostic.md``.)
- ****RSM:**** Multi-platform review-acquisition system; route patients to Yelp + Google + Healthgrades, not just Google. (See ``mountainsidersm_clarity_diagnostic.md``.)
- ****Lake Forest:**** Hero, meta description, and GBP rewrite leading with same-day care + Harvard implant credentials. (See ``mountainsidelakeforest_clarity_diagnostic.md``.)
- Roll out real-time booking to the four locations not in the pilot, in order: Yucaipa, Rancho Mirage, Lake Forest, RSM.

Ongoing

- Monthly blog cadence (already happening — keep it going).
- Monthly GBP posts at all five locations on a coordinated network template.
- Monthly review velocity check at each location.
- Quarterly Maps ranking audit on the five key search terms in each market.
- Quarterly photo refresh per location (one 30-minute session per quarter, rotated through the five offices).

The Next Step

Everything in this audit is yours to implement. The fixes are spelled out, in order of impact. Most practices won't get to them — not because they don't matter, but because there's never time. If you'd rather have someone run them for you, the Clarity Audit is the next step. We'll go deeper on the things that need it, prioritize against your specific goals, and give you a 90-day plan you can hand off. No pressure either way — the audit you're holding is the audit, not the appetizer.

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